VENLAFAXINE

Raises BP

Severe discontinuation

CHEMICAL NAME (RS)-1-[2-(dimethylamino)-1-(4-methoxyphenyl)ethyl]cyclohexanol (hydrochloride)

CLASS Serotonin–noradrenaline reuptake inhibitor (SNRI)

TRADE NAMES Efexor XL · Effexor · Venlalic · Vensir

THERAPEUTIC USES / INDICATIONS

- Major depression (incl. treatment-resistant).
- Generalised anxiety disorder, social anxiety, panic disorder.
- Off-label: hot flushes, neuropathic pain.

ADMINISTRATION

- **PO** MR once daily with food (swallow whole) or IR twice daily.

DOSAGES

- **Depression:** 75 mg daily (MR), increase to 150–225 mg; max 375 mg (specialist).
- **GAD:** 75 mg daily.
- **Renal/hepatic impairment:** reduce by 25–50%.

PHARMACOKINETICS

- **Onset:** 2–4 weeks.
- Metabolised by CYP2D6 to active desvenlafaxine.
- **t½:** 5 h (metabolite 11 h) → withdrawal common.

NURSING MANAGEMENT

- Monitor BP at baseline and dose increases.
- Mood/suicide risk.
- Plan slow taper.

PHARMACODYNAMICS

- Inhibits **5-HT reuptake** (low dose) and **noradrenaline reuptake** (higher doses >150 mg) → ↑ both monoamines.

SIDE & ADVERSE EFFECTS

SIDE EFFECTS (COMMON)

- Nausea, dizziness, dry mouth, sweating.
- Insomnia, headache.
- Sexual dysfunction.

ADVERSE EFFECTS (SERIOUS)

- Dose-related hypertension.
- Discontinuation syndrome (severe).
- Suicidality; serotonin syndrome; hyponatraemia; bleeding.
- Arrhythmias in overdose (more toxic than SSRIs).

CONTRAINDICATIONS & CAUTIONS

- MAOIs; uncontrolled hypertension (relative); high arrhythmia risk.

KEY INTERACTIONS

- **Serotonergic drugs:** serotonin syndrome.
- **Antihypertensives:** BP effects.
- **Anticoagulants, NSAIDs:** bleeding.

PATIENT EDUCATION

- Take with food; swallow MR whole.
- Have BP checked regularly.
- Do not stop suddenly.
- Report worsening mood.

KEY POINTS

- SNRI; 5-HT + NA reuptake inhibitor.
- Raises BP; severe withdrawal.
- 75 mg MR daily.
- More dangerous in overdose than SSRIs.